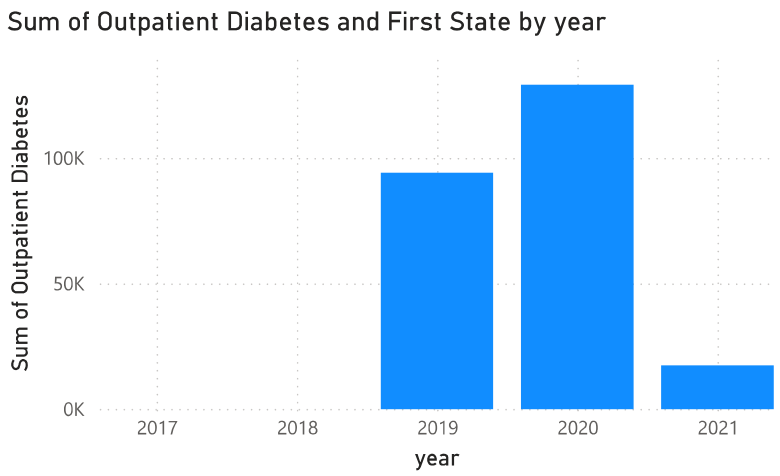
Kerala Death counts by states

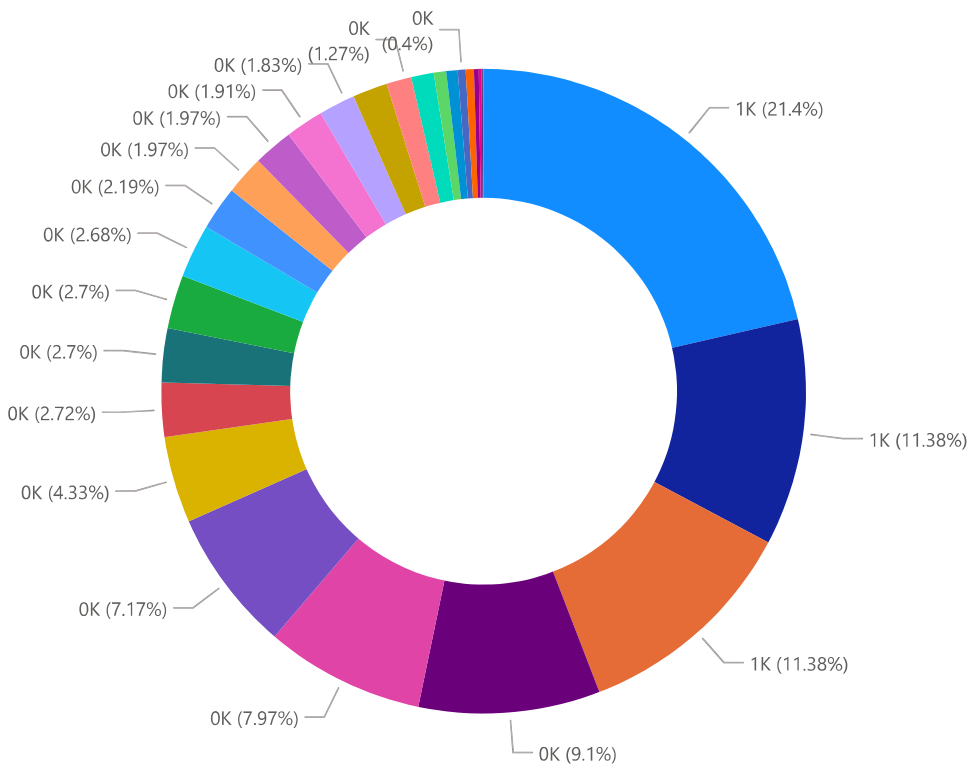
year	State	Sum of Outpatient Diabetes	Sum of Male Hiv Number Positive	Sum of Deaths Following Female Sterilization	Sum of Deaths Following Male Sterilization	Sum of Infant Deaths Up to 4 W
2019	Kerala	12078	5	14	317	
Total		**12078**	**5**	**14**	**317**	



Next

State wise Emergency death reports

Sum of Number of Deaths Occurring at Emergency Department by State

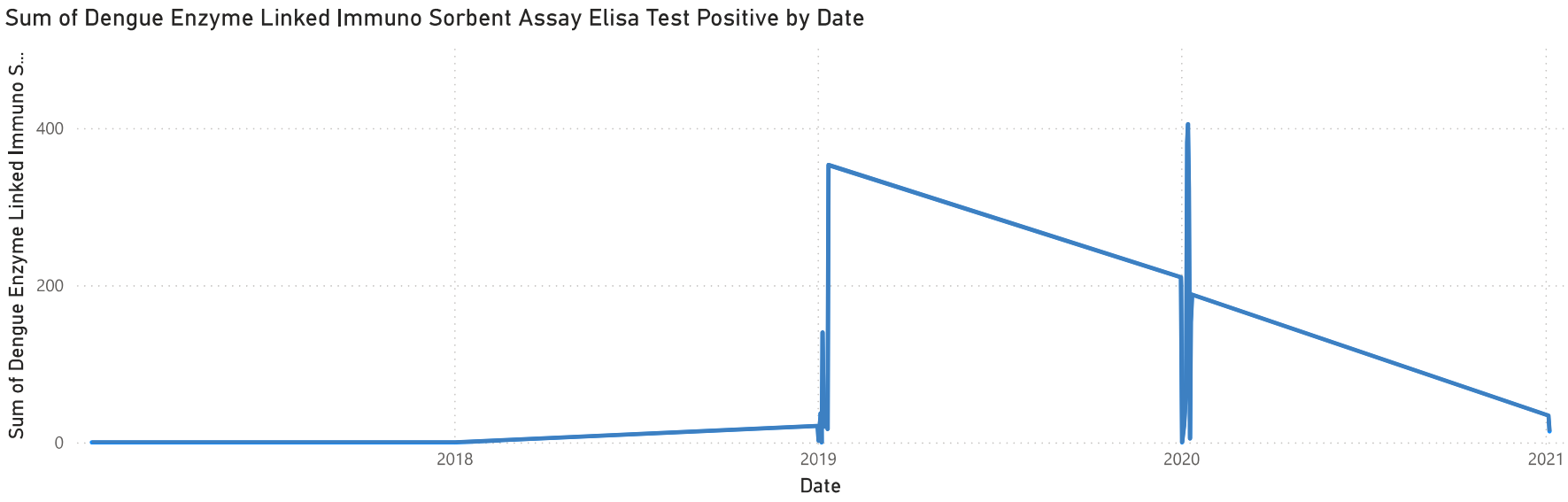


State

- Uttar Pradesh
- Delhi
- Madhya Pradesh
- Bihar
- Andhra Pradesh
- Odisha
- Rajasthan
- Assam
- Gujarat
- Tamil Nadu
- Haryana
- Karnataka
- Maharashtra
- West Bengal
- Chhattisgarh
- Jharkhand
- Kerala
- Telangana
- Punjab
- Uttarakhand

Next

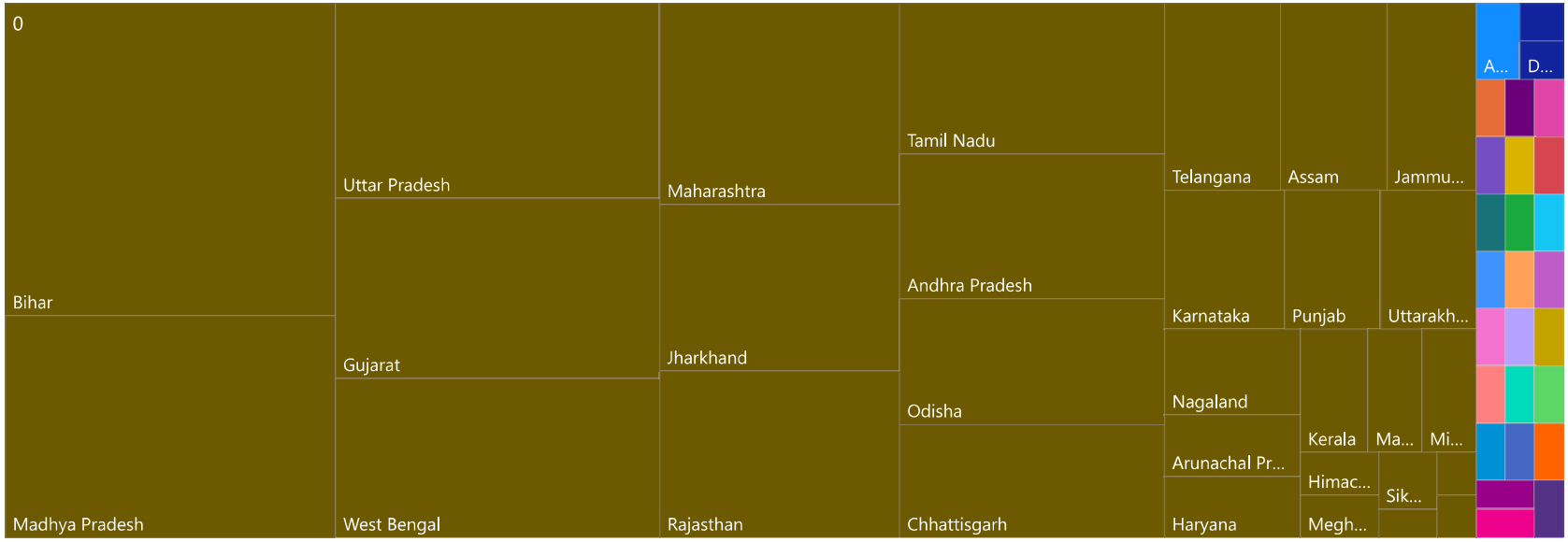
Year-wise analysis of Dengue cases



Next

Live birth analysis by states

Count of State by Live Birth Female and State



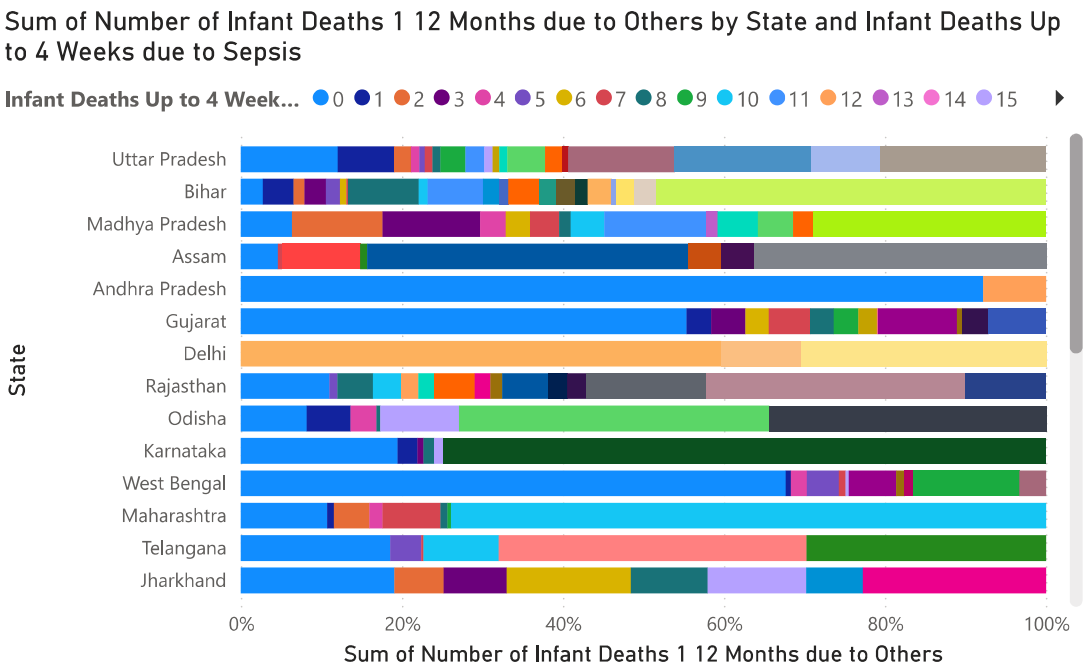
Next

Geographical analysis of HIV positive

Sum of Male Hiv Number Positive by State and Female Non Anc Hiv Number Positive

Next

State-level infant death statistics



State Code , Sub-District Code

- 1
- 2
- 3
- 5
- 6

Next

HIV Testing and Cases Overview

State , Sub District Name

- Andhra Pradesh
- Arunachal Pradesh
- Assam
- Bihar
- Chhattisgarh
- Delhi
- Gujarat
- Haryana
- Himachal Pradesh

21K
Sum of Female Non Anc Hiv Number Positive

103K
Sum of Female Non Anc Hiv Number Tested

3211
Sum of Male Hiv Number Tested

251
Sum of Male Hiv Number Positive

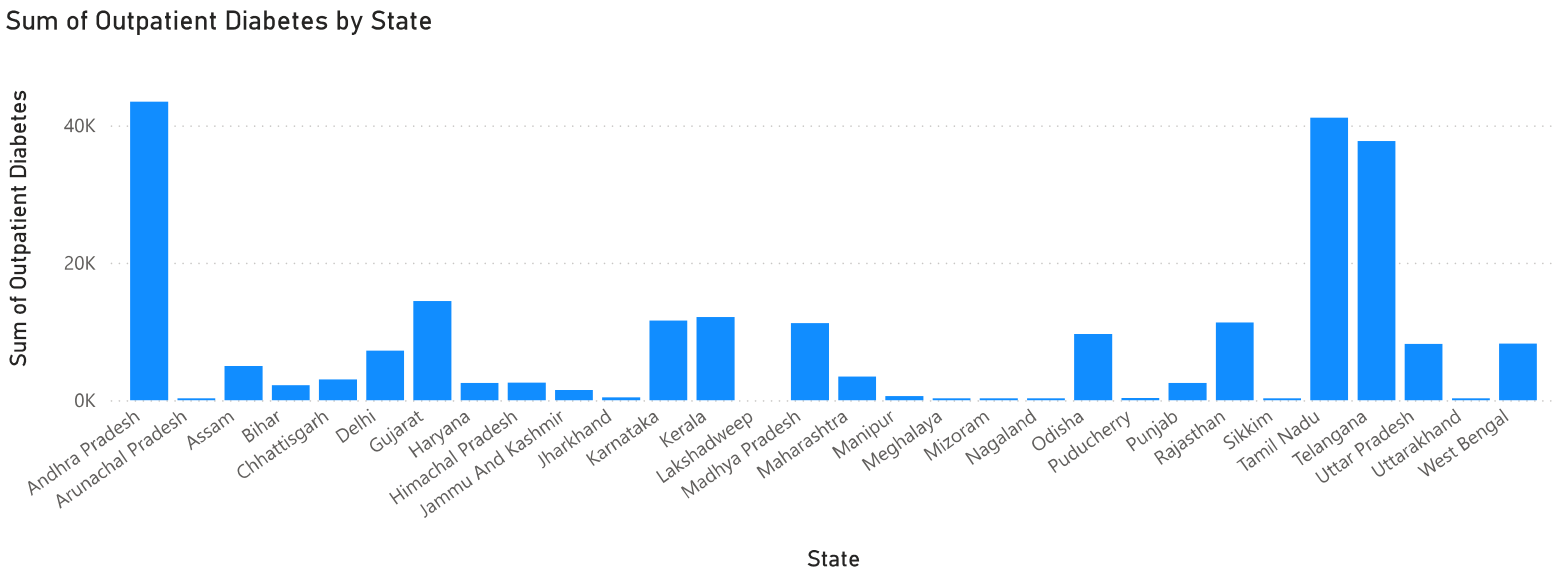
129
Sum of Number of Pregnant Women Screened for Hiv

1968
Sum of Number of Adolscent Adult Deaths due to Hivaids

174K
Sum of Out of The Above Number Screened Positive Number Confirmed With Hiv Infection at Integrated Couns...

Next

State-wise Distribution of Diabetes Patients



State , Sub Distric...

- Andhra Prade...
- Arunachal Pra...
- Assam
- Bihar
- Chhattisgarh
- Delhi
- Gujarat
- Haryana
- Himachal Pra...
- Jammu And K...
- Jharkhand
- Karnataka
- Kerala
- Lakshadweep
- Madhya Prad...
- Maharashtra

Next

Conclusion

1. **HIV Screening Coverage**
   - A large number of individuals have been tested for HIV, especially female Non-ANC groups, indicating strong screening initiatives.
   - However, the number of confirmed HIV cases remains high, suggesting that HIV is still a significant public health concern.
2. **Gender Gap in HIV Testing**
   - HIV testing among women is significantly higher than men, possibly due to maternal health programs.
   - This indicates that male testing coverage may need improvement.
3. **Regional Disparities in Health Outcomes**
   - Some states contribute disproportionately to infant deaths and HIV cases.
   - This shows unequal access to healthcare services across states.
4. **Infant Mortality Patterns**
   - A large proportion of infant deaths occur within the first four weeks, highlighting the importance of neonatal healthcare services.
5. **Population Influence on Health Indicators**
   - Highly populated states show higher birth rates, higher disease cases, and more health-related events, increasing the burden on healthcare systems.

Recommendations

Increase HIV Awareness and Testing for Men
- Encourage HIV screening programs targeting male populations, particularly in high-risk areas.

Strengthen Neonatal Healthcare
- Improve postnatal care, neonatal intensive care units (NICUs), and early disease detection to reduce infant mortality.

Target High-Risk Regions
- Focus government health programs on states with higher HIV prevalence and infant mortality rates.

Improve Maternal and Child Health Services

Year-wise Analysis of Dengue Cases

Key Observations

- Dengue cases were very low before 2018.
- A sharp increase occurs in 2019, indicating a possible outbreak or improved detection.
- Peak observed around 2020 with the highest number of positive ELISA tests.
- A declining trend in 2021, suggesting better control or seasonal variation.

Insight

- Dengue shows a cyclical outbreak pattern, peaking in certain years.
- The spike in 2019–2020 may indicate:
  - Increased transmission
  - Better testing/reporting systems
- The decline afterward suggests effective public health interventions or seasonal drop.

State-wise Emergency Death Reports

Key Observations

- One state contributes around 21% of total emergency deaths, the highest share.
- A few major states (like Delhi, Madhya Pradesh, Bihar) contribute significant portions (~9–11%).
- Many states have very small shares (<3%), indicating uneven distribution.

Insight

- Emergency-related deaths are highly concentrated in a few states.
- This could be due to:
  - Higher population
  - Poor emergency response systems
  - Limited hospital infrastructure

Kerala Death Counts (Tabular Data)

Key Observations

- Data includes district-level details like Kollam, Kannur, Thrissur, Ernakulam, etc.

Overall Combined Insights

1. Disease Trend Patterns
   - Dengue cases show spikes during specific years, indicating outbreak patterns.
   - Continuous monitoring is essential to predict future outbreaks.
2. Regional Inequality in Healthcare
   - Emergency deaths and general health metrics are not evenly distributed across states/districts.
   - Some regions require urgent healthcare improvements.
3. Importance of District-Level Analysis
   - State-level data alone is not enough.
   - District-level insights (like in Kerala) reveal hidden patterns and disparities.
4. Rural Healthcare Significance
   - Rural areas show considerable health activity and counts, emphasizing:
     - Need for better rural infrastructure
     - Improved accessibility to healthcare services

Final Insight

- "The analysis highlights temporal spikes in dengue cases, regional disparities in emergency deaths, and significant variation in district-level health data. The findings emphasize the need for targeted interventions, especially in high-burden states and rural areas, along with continuous monitoring to manage disease outbreaks effectively."